

Sutter Urgent Care — Oakland Telegraph

3838 Telegraph Ave, Oakland, CA 94609 · (510) 555-0188

Date of Visit: 05/12/2024 14:32
Patient: PARK, DAVID **DOB:** 09/24/1986 **Sex:** M
MRN-SUTTER: 3349812
Provider: Jennifer Okonkwo, PA-C

PRESENTING PROBLEM

Acute low back pain, onset this morning.

HISTORY OF PRESENT ILLNESS

Pt is a 38yo M presenting with acute onset low back pain that began this morning while moving boxes during a household move. Reports a "pulling" sensation in the lumbar region followed by sharp pain when straightening up. Pain rated 7/10 at worst, 5/10 at rest. Localized to the right lumbar paraspinal area, no radiation to buttocks or legs. No numbness, no tingling, no bowel or bladder dysfunction. No prior history of back pain.

Patient took ibuprofen 600 mg approximately 3 hours prior to arrival with mild relief. He is able to ambulate but with obvious discomfort. Concerned because he has a long flight scheduled in 4 days.

PMH: none significant. Medications: none chronic. Allergies: NKDA. Social: occasional alcohol, no tobacco, no recreational drugs.

EXAMINATION

Vitals: BP 128/82, HR 76, T 98.0F, SpO2 99% RA.

General: well-appearing, in mild discomfort.

Back: tenderness to palpation over right L4-L5 paraspinal musculature. No midline spinous tenderness. ROM limited by pain, particularly forward flexion (~40 degrees).

Neuro: strength 5/5 bilateral lower extremities. Sensation intact L2-S1 dermatomes bilaterally. DTRs 2+ patellar and Achilles bilaterally. Negative straight-leg raise bilaterally.

No red flag features.

ASSESSMENT

Acute mechanical low back pain (M54.5), most likely lumbar paraspinal muscle strain. No red flags on history or exam.

PLAN

- 1. Ibuprofen 600 mg PO q6h PRN, with food. Continue for 5-7 days.
 - 2. Cyclobenzaprine 5 mg PO TID PRN muscle spasm, for 5 days. Counseled re: drowsiness, no driving while taking.
 - 3. Activity modification: avoid heavy lifting, twisting. Walk as tolerated. No strict bed rest.
 - 4. Apply heat 15-20 min, several times daily.
 - 5. Follow up with PCP in 2 weeks if not improved. Return to ED for new neurologic symptoms, bowel/bladder changes, or pain not controlled with medications.
- Counseling: 10 min on natural history of acute LBP and red flag symptoms requiring urgent return.

— Jennifer Okonkwo, PA-C
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